

Children's Community Audiology Service Referral Form

Childrens.commaudiologyreferrals@mpft.nhs.uk

East Locality (Burton, Tamworth, Lichfield, Uttoxeter): Tel: 01283 505168

West Locality (Stone, Stafford, Cannock, Rugeley, South Staffordshire): Tel: 01785 221664

All fields are MANDATORY and must be completed, otherwise the form will be returned to the referrer

Please refer to the referral guidelines below.

PERSONAL DETAILS	
Patient Name: Given Name Surname	Gender: Gender(full)
D.O.B: Date of Birth	NHS No: NHS Number
Address: Home Full Address (single line)	Mobile: Landline: Patient Home Telephone E-mail address: PLEASE NOTE: By completing these details you are consenting to being contacted via email or mobile / text message
Postcode: Home Address Postcode	Ethnicity: Ethnic Origin
	Is an interpreter required? ☐ YES ☐ NO If so, which language?
GP DETAILS	
Name: Registered GP Full Name Address: Organisation Name Organisation Full Address (single line)	Telephone number: Organisation Telephone Number Secure e-mail address: Organisation E-mail Address
Postcode: Organisation Postcode	
CONSENT	
Names of all with parental responsibility and their relationship to the child, plus address/s if different from above:	
Has someone with parental responsibility consented to this referral? ☐ YES ☐ NO	
Print Name/s: Signed:	Relationship/s: Verbal Consent Given: ☐

RISKS	
Other agencies involved? Social worker, health professional? ☐ YES ☐ NO <i>If so, please provide details (i.e. name, address and contact number)</i>	Safeguarding information: <i>Please provide details (i.e. name, address and contact number)</i>
Risk factors for Targeted Referral (For referral guidelines please see below) ☐ High risk of middle ear problems (e.g Downs syndrome/cleft palate) (1) ☐ Craniofacial anomalies and syndromes (2) ☐ Passed AABR but not OAE (NICU/SCBU babies) (3) ☐ Confirmed congenital infection (TORCH) (4)	
Is child's development age appropriate? ☐ YES ☐ NO <i>If so, please provide details</i>	
Special educational needs? ☐ YES ☐ NO <i>If so, please provide details</i>	
Speech therapy involvement? ☐ YES ☐ NO <i>If so, please provide details</i>	
Any known diagnosis/medical conditions? ☐ YES ☐ NO <i>If so, please provide details</i>	
Ear infections? ☐ YES ☐ NO <i>If so, please provide details</i>	
REASON FOR REFERRAL (see referral guidelines)	
Main reason for referral Whatever main referral reason please tick all areas above or below that are relevant as this information is useful to those processing referrals. PLEASE read guidelines to aid appropriate referral. ☐ Parental/Professional concern (5) ☐ Family history early onset hearing loss (not glue ear) (6) ☐ Neuro-developmental concerns (7) ☐ Amino-glycosides (8) ☐ Meningitis/skull fracture (9)	
ADDITIONAL INFORMATION (see referral guidelines)	
Please include any additional information, particularly if family history or parental / professional concerns. Please give as much detail as possible. 	
Referred by: Free Text Prompt Designation:	Referrer contact address: Organisation Name Organisation Full Address (single line)

Signed: Date of referral: Short date letter merged	Postcode: Organisation Postcode Telephone number: Organisation Telephone Number E-mail: Organisation E-mail Address
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Please see next page for Children's Community Audiology Referral Guidelines

REFERRAL GUIDELINES

We are a community children's audiology service covering South Staffordshire. We run clinics at Coach House, St Michael's Court, Lichfield and The Bridge, St George's Parkway, Stafford. We provide hearing tests for children aged 6 months -18 years who have hearing difficulties including those with middle ear disorders and complex needs. Where clinically necessary we may refer children to a tertiary audiology service, an ear, nose and throat surgeon (ENT) or their GP.

1. HIGH RISK OF MIDDLE EAR PROBLEMS INCLUDES: Downs Syndrome, Cleft palate.

2. CRANIO-FACIAL ANOMALIES OR SYNDROME INCLUDES: Chromosomal or syndromic conditions related to a hearing loss, Branchial arch and cervical spine anomalies.

3. NICU/SCBU baby with Bilateral No Clear Response at OAE, but Bilateral Clear Response at AABR

4. CONFIRMED CONGENITAL INFECTION due to Toxoplasmosis, Rubella or Cytomegalovirus (TORCH)

5. PARENTAL or PROFESSIONAL CONCERN

EXAMPLES:

- **Severe** Language Delay (No words by 2 years old). **It is not necessary to routinely refer a child with delayed speech, ONLY if a hearing loss is suspected.**
- Recurrent ear infections where there is felt to be significant effect on hearing.
- Behaviour changes that appear to be linked to hearing (poor response, decreased attention span, etc.)

6. FAMILY HISTORY permanent/sensorineural hearing loss in parents and siblings only; should be permanent (i.e. not glue ear) and present from early childhood irrespective of the degree of loss (Only need to refer if hearing concerns. However it is advisable to arrange a hearing screen with the school nurse at 4/5 years to ensure satisfactory hearing).

7. NEURO-DEGENERATIVE or NEURO DEVELOPMENTAL DISORDERS i.e. cases of developmental delay associated with a neurological disorder. Where there are hearing concerns.

8. AMINOGLYCOSIDES The prescribing Clinician has identified that the levels have risen outside the therapeutic range (or if full records are unavailable.)

9. BACTERIAL MENINGITIS OR MENINGOCOCCAL DISEASE Confirmed or suspected (**should be referred and seen as soon as possible after acute illness settles preferably within 4 weeks of hospital discharge**). Also refer if there has been a **SKULL FRACTURE** of the Temporal Bone.

10. Jaundice where the bilirubin level (normally unconjugated) reached a level indicating need for an exchange transfusion.

Reference:

Newborn Hearing Screening Programme (NHSP): Guidelines for screening providers, commissioners and public health for changes in targeted follow up procedures.

Davies, A, Wood, S, and Sutton, G. <http://hearing.screening.nhs.uk>